

Evaluation and Management (E/M) Services - 2024 Policy Update

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BACKGROUND

This article summarizes CMS E/M policy changes effective January 1, 2024, reflecting the 2024 Medicare Physician Fee Schedule final rule.

SECTION 1 - SCOPE AND COVERAGE

Governs reimbursement for CPT codes 99202-99215 for outpatient and telehealth visits.

Covered: in-network physicians, mid-level practitioners, licensed telehealth providers.

Telehealth: requires HIPAA-compliant real-time audio-visual platform.

Audio-only (Modifier 93): allowed with prior authorization when video not feasible.

SECTION 2 - DOCUMENTATION REQUIREMENTS (2024 UPDATE)

Documentation must be based on ONE of two methods:

METHOD A - Medical Decision Making (MDM):

Straightforward: minimal problems, minimal data, minimal risk.

Low: low complexity, limited data, low risk.

Moderate: moderate complexity, moderate data, moderate risk.

High: high complexity, extensive data, high risk.

METHOD B - Total Time on Date of Encounter:

Includes face-to-face AND non-face-to-face time on the date of service.

Does NOT include time on previous or subsequent days.

KEY CHANGES:

The three-key-component method is NO LONGER REQUIRED.

Copy-forward documentation PROHIBITED without explicit attestation.

Missing attestation = AUTOMATIC CLAIM DENIAL.

SECTION 3 - CPT CODE GUIDELINES

3.1 NEW PATIENT VISITS (no prior face-to-face within 3 years):

99202 - Straightforward | 15-29 min

99203 - Low | 30-44 min

99204 - Moderate | 45-59 min

99205 - High | 60-74 min

3.2 ESTABLISHED PATIENT VISITS:

99211 - Minimal | May not require physician

99212 - Straightforward | 10-19 min

99213 - Low | 20-29 min

99214 - Moderate | 30-39 min

99215 - High | 40-54 min

3.3 TELEHEALTH VISITS (NEW - January 1, 2024):

Same CPT codes 99202-99215 with telehealth place of service.

POS 02: Patient not in home. POS 10: Patient in home.

Modifier 95: REQUIRED on all telehealth E/M claims.

Modifier 93: Audio-only (prior authorization required).

3.4 ADD-ON CODE G2211:

Billable with most E/M visits for ongoing care complexity.

Cannot bill with Modifier 25 on same claim.

Reimbursement: approx. \$16.57 per claim.

SECTION 4 - BILLING RESTRICTIONS AND DENIAL TRIGGERS

CLAIM DENIAL will result from:

1. Copy-forward documentation without attestation.
2. Telehealth claims missing Modifier 95.
3. Audio-only (Modifier 93) without prior authorization.
4. G2211 billed with Modifier 25 on same claim.
5. Split/shared visit where billing provider did not perform >50% of time.

Additional:

Modifier 25 required when E/M billed same-day as minor procedure.

Incident-to billing NOT covered for new patients.

SECTION 5 - REIMBURSEMENT RATES (2024)

Participating providers: 115% of MPFS (increased from 110%).

Non-participating providers: 80% of MPFS (unchanged).

Telehealth parity with in-person visits through December 31, 2024.

CY 2024 Conversion Factor: \$32.74 (down from \$33.89 in CY 2023).

SECTION 6 - EXCLUDED SERVICES

Telephone-only consultations without in-office or telehealth component.

Services by unlicensed personnel.

Administrative encounters without clinical evaluation.

Telehealth via non-HIPAA platforms (e.g., FaceTime, Zoom without BAA).

G2211 billed concurrently with Modifier 25.

REFERENCES

1. CY 2024 PFS Final Rule. Federal Register Vol. 88, No. 220 (Nov 16, 2023).
2. AMA CPT Professional Edition 2024.
3. CMS MLN Booklet: Evaluation and Management Services (MLN006764).
4. CMS Medicare Claims Processing Manual, Chapter 12, Section 30.6.